

Wardle Editorial Canon

Standards for Governed Editorial Artifacts

Part IV - Diagnosis Education Standards

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1. Purpose

Diagnosis Education Standards define how diagnosis-level learner education is structured, authored, generated, revised, evaluated, approved, published, and maintained within the Wardle Editorial Operating System.

Diagnosis Education is the governed learner-facing explanation of a diagnosis outside the progression of any single clinical case.

Its purpose is to help learners consolidate the clinical pattern, interpret important findings, distinguish mimics, choose investigations, understand management priorities, avoid common errors, and retain the knowledge through active recall.

Diagnosis Education must therefore be more than a collection of correct medical facts.

It must be clinically defensible, educationally intentional, diagnosis-specific, internally coherent, appropriately evidenced, and aligned with the approved educational foundation of the diagnosis.

These standards govern the complete Diagnosis Education artifact and every section or recall unit contained within it.

2. Diagnosis Education as a Governed Artifact

Diagnosis Education is a first-class governed editorial artifact associated with one Diagnosis Registry identity.

It translates approved diagnosis-level educational intent into a structured resource that learners can study after gameplay, during review, or through recall practice.

Diagnosis Education may include:

- Summary;
- Clinical Pattern;
- Key Symptoms;
- Key Signs;
- Examination Pearls;

- Scoring Systems or Diagnostic Criteria;
- Investigations;
- Differentials;
- Management;
- Complications;
- Pitfalls;
- Recall Prompts;
- References.

The artifact is governed as a coherent whole, while individual sections may also possess their own drafts, reviews, provenance, and revision history.

A section that is individually acceptable must still remain consistent with the rest of the Diagnosis Education artifact.

3. Foundational Distinctions

3.1 Diagnosis Education and Case Explanation

A Case Explanation explains the reasoning within one Clinical Case.

Diagnosis Education explains the diagnosis beyond that case.

A Case Explanation may discuss why particular clues changed probability, how a mimic was weakened, and where diagnostic lock-in occurred. Diagnosis Education should generalise governed knowledge across clinically appropriate presentations without implying that one case represents the entire disease.

Diagnosis Education should not copy a case explanation and remove the patient details. It should provide a diagnosis-level learning resource with broader scope, explicit structure, and controlled clinical generalisation.

3.2 Educational Intent and Learner-Facing Education

The Editorial Brief, Learning Goals, Teaching Rules, required mimics, discriminators, investigation anchors, management anchors, and education guidance define what Wardle intends to teach.

Diagnosis Education communicates that intent to learners.

Learner-facing education should be evaluated against approved intent. It should not create new educational priorities accidentally or contradict the educational foundation from which it was produced.

3.3 Diagnosis-Level Knowledge and Case-Specific Knowledge

Diagnosis-level knowledge describes principles that remain defensible across an appropriately defined diagnosis scope.

Case-specific knowledge describes the interpretation of one presentation, severity, population, setting, or reasoning path.

Diagnosis Education may use examples, but it must not generalise a finding, investigation result, or management choice beyond the context supported by clinical knowledge and evidence.

3.4 Section and Artifact

A Diagnosis Education section is a governed component of the complete Diagnosis Education artifact.

Section-level review may determine whether a section is acceptable in isolation. Artifact-level review must also determine whether sections agree, use compatible scope, avoid duplication, and form a coherent learner resource.

3.5 Correctness and Educational Usefulness

A statement may be medically correct yet educationally weak because it is generic, irrelevant to the target learner, poorly prioritised, unsupported by context, or disconnected from diagnostic reasoning.

Diagnosis Education requires both clinical correctness and educational usefulness.

3.6 Reference and Evidence Authority

A reference identifies an external source available to support or extend a claim.

An Evidence Relationship records how a governed claim is supported within the WEOS knowledge system.

The presence of a reference does not independently establish that every statement in a section is correct, applicable, or institutionally approved.

4. Educational Purpose and Scope

Every Diagnosis Education artifact must have a defined educational purpose.

Its scope should be derived from the diagnosis identity, learner level, editorial brief, curriculum role, and intended Wardle use.

The educational purpose should identify:

- what learners should understand after studying the resource;
- which diagnostic pattern they should recognise;
- which mimics they should distinguish;
- which investigations they should interpret;
- which management or escalation principles they should retain;
- which misconceptions or unsafe shortcuts the education should prevent;
- which concepts should be reinforced through recall.

Scope should be sufficient to support diagnostic education without attempting to reproduce a comprehensive specialty textbook.

Depth should be determined by educational need rather than by the maximum amount of content that can be generated.

5. Artifact Identity and Diagnosis Relationship

5.1 Stable identity

Every Diagnosis Education artifact must possess a stable identity independent of its current wording, section content, or publication status.

Stable identity enables revision history, review continuity, publication history, recall linkage, knowledge extraction, supersession, and auditability.

5.2 Diagnosis mapping

Every Diagnosis Education artifact must resolve to one governed Diagnosis Registry entry.

The diagnosis relationship must identify the canonical diagnosis rather than infer identity from a learner-facing title or generated summary.

Education with unresolved diagnosis identity must not become approved or published.

5.3 Scope qualifiers

Where the education applies only to a subtype, stage, age group, pregnancy state, setting, or severity, that scope must be explicit.

Scope qualifiers must not be hidden in one section while other sections imply universal applicability.

6. Source and Provenance

Diagnosis Education may originate through manual authoring, AI generation, section generation, administrative revision, migration, restoration, evidence-led maintenance, or controlled adaptation from earlier educational material.

Source describes origin. It does not establish quality or authority.

The artifact and each material revision should preserve:

- original producer;
- production method;
- generation or authoring purpose;
- diagnosis context;
- source Editorial Brief;
- source Learning Goals and Teaching Rules;
- source artifacts and versions;
- evidence context;
- review history;
- application history where a draft was accepted and applied.

Provenance should make it possible to reconstruct why a section exists, what context shaped it, and how it became governed learner-facing knowledge.

7. Diagnosis Education Revision

7.1 Revision purpose

A Diagnosis Education Revision is a versioned snapshot of the complete learner-facing education artifact at a particular point in editorial history.

Approval and publication must apply to an identified revision rather than to an indefinitely mutable record.

7.2 Material change

A new revision should be created when a material change affects:

- clinical meaning;
- diagnostic scope;
- educational emphasis;
- section structure;
- investigation guidance;
- differential reasoning;
- management guidance;
- scoring criteria;
- complications;
- high-risk pitfalls;
- recall answers;
- references supporting material claims.

Minor typographical correction may follow implementation policy, but no correction should erase a previously reviewed or published state.

7.3 Revision comparison

Reviewers should be able to determine what changed, why it changed, which source issue prompted the change, whether the change affects other sections, and which review must be repeated.

Approval of an earlier revision does not automatically approve a materially changed revision.

8. Canonical Structure

The canonical Diagnosis Education structure is modular but coherent.

Not every diagnosis requires identical depth in every section. However, section omission must be deliberate and explainable rather than the result of incomplete generation or authoring.

Section	Primary educational question
Summary	What is this diagnosis and why does it matter?
Clinical Pattern	What presentation should the learner recognise?
Key Symptoms	Which reported features meaningfully affect suspicion?
Key Signs	Which observable findings support, discriminate, or indicate severity?
Examination Pearls	How should examination be performed and interpreted?
Scoring Systems	Which criteria or scores assist diagnosis, severity, or action?
Investigations	What should be tested, what findings are expected, and what are the limitations?
Differentials	Which diagnoses are confused with this one and how are they separated?
Management	What management, escalation, and follow-up principles are educationally essential?
Complications	What important consequences should be recognised or prevented?
Pitfalls	Which reasoning or management errors should the learner avoid?
Recall Prompts	Which governed concepts should the learner retrieve actively?
References	Which external sources support or extend the education?

Implementations may present these sections under learner-friendly labels, but the canonical editorial meaning must remain preserved.

9. Summary

9.1 Purpose

The Summary provides a concise diagnosis-level orientation.

It should establish the diagnosis identity, essential clinical meaning, typical pattern, and educational importance without attempting to duplicate every later section.

9.2 Required qualities

A strong Summary should be:

- diagnosis-specific;
- clinically accurate;
- brief enough for orientation;
- consistent with the diagnosis scope;
- free from decorative language;
- free from unsupported certainty;
- aligned with the approved Learning Goals.

Where a concise definition is educationally necessary, it should be incorporated into the Summary or represented as a clearly governed component rather than generated as isolated dictionary text.

9.3 Prohibited patterns

The Summary should not:

- repeat the canonical name without explaining the clinical concept;
- use a case narrative as the diagnosis definition;
- present one manifestation as universal;
- include detailed management that belongs later;
- rely on vague phrases such as “a condition with several symptoms”;
- make epidemiological or prognostic claims without appropriate support.

10. Clinical Pattern

10.1 Purpose

The Clinical Pattern describes the recognisable configuration of context, chronology, symptoms, signs, investigations, and severity that should organise learner understanding.

It should teach pattern recognition without reducing diagnosis to a memorised slogan.

10.2 Content standard

The Clinical Pattern may identify:

- typical patient or risk context;
- onset and time course;
- dominant symptom pattern;
- important examination pattern;
- expected investigation pattern;
- features indicating severity or complication;
- common atypical or population-specific variation where educationally important.

The section should distinguish between common, characteristic, supportive, and pathognomonic findings where those distinctions matter.

10.3 Avoiding overfitting

The Clinical Pattern must not overfit to the presentation used in one Wardle case.

A learner should not leave the section believing that the diagnosis requires every clue used in a flagship case or that absence of one classic feature excludes the diagnosis.

11. Key Symptoms

11.1 Purpose

Key Symptoms identify patient-reported features that meaningfully contribute to diagnosis, severity assessment, differential discrimination, or escalation.

11.2 Symptom structure

A symptom entry should communicate more than a label.

Where relevant, it should identify:

- quality;
- location;
- timing;
- duration;
- progression;
- triggers;
- relieving factors;
- associated symptoms;
- clinical significance;
- which mimic it helps distinguish.

Symptoms should be prioritised. Long undifferentiated lists weaken recognition and obscure the diagnostic pattern.

11.3 Negative symptoms

Important absent symptoms may be included where their absence meaningfully changes probability or helps distinguish a dangerous mimic.

Negative findings should not be presented as absolute exclusion rules unless clinically justified.

12. Key Signs

12.1 Purpose

Key Signs identify observable examination or bedside findings that support diagnosis, discriminate alternatives, indicate severity, reveal complications, or change management.

12.2 Sign interpretation

A sign should be connected to its clinical meaning.

Education should explain whether the sign is:

- common;
- characteristic;
- late;
- severity-related;
- a complication signal;
- a discriminator;
- population-dependent;
- limited by poor sensitivity or specificity.

A correct list of signs without interpretation does not satisfy the educational standard.

13. Examination Pearls

13.1 Purpose

Examination Pearls teach how to look for, elicit, interpret, or avoid missing clinically meaningful examination findings.

They should strengthen bedside reasoning rather than repeat the Key Signs list.

13.2 Appropriate content

An Examination Pearl may address:

- examination sequence;
- technique;
- comparison with the opposite side;
- patient positioning;
- interpretation of a subtle finding;
- limitations of a sign;
- a finding that changes urgency;
- a commonly omitted part of examination.

Technique should not be described in procedural detail beyond the educational scope or in a way that creates unsafe unsupervised practice.

14. Scoring Systems and Diagnostic Criteria

14.1 Purpose

This section governs formal scores, classifications, staging systems, and diagnostic criteria used to support diagnosis, severity assessment, prognosis, or management decisions.

14.2 Required elements

A scoring-system entry should identify:

- official or accepted name;
- intended purpose;
- applicable population and setting;
- components;
- interpretation;
- validated use;
- important limitations;
- action thresholds where appropriate;
- source or authority;
- version where criteria have changed.

A score should not be included merely because it is associated with the diagnosis.

Its educational value and correct use must be explicit.

14.3 Safety and fidelity

Criteria, cut-offs, units, and point assignments must be reproduced accurately and verified against an appropriate source.

WEOS must not invent, approximate, combine, or silently simplify scoring systems in a way that changes interpretation.

Where a score does not replace clinical judgement, that limitation should be stated clearly.

14.4 Mnemonics and memory aids

Mnemonics are not a required canonical Diagnosis Education section.

Where used, a mnemonic should support accurate recall of governed content, avoid offensive or misleading language, and not replace understanding of clinical meaning.

A mnemonic that omits important qualifiers or creates an unsafe rule should not be published.

15. Investigations

15.1 Purpose

The Investigations section teaches what to investigate, why the investigation is used, what findings are expected, how results should be interpreted, and what limitations remain.

15.2 Investigation hierarchy

Investigations should be organised according to clinical purpose rather than presented as an unprioritised list.

Possible purposes include:

- initial bedside assessment;
- confirmatory testing;
- excluding dangerous alternatives;
- severity assessment;
- complication assessment;
- pre-treatment assessment;
- monitoring;
- specialist or second-line investigation.

15.3 Result interpretation

Where educationally relevant, each investigation should identify:

- expected finding;
- diagnostic role;
- sensitivity or specificity limitations;
- timing;
- false-negative or false-positive context;
- population limitations;
- how the result changes probability or action.

A test result should not be represented as diagnostic in isolation where clinical interpretation remains necessary.

15.4 Imaging and pathology

Imaging and pathology guidance should use accurate descriptive terminology and distinguish a supportive appearance from a definitive diagnosis.

Image acquisition, specialist reporting, and clinical correlation limitations should be stated where necessary.

16. Differentials

16.1 Purpose

The Differentials section teaches which diagnoses are plausibly confused with the target diagnosis, why the confusion occurs, and how they should be distinguished.

It should support diagnostic reasoning rather than provide a generic list of alternatives.

16.2 Differential entry standard

Each important differential should identify:

- resolved Diagnosis Registry identity where available;
- shared presentation;
- reason for confusion;
- finding or context favouring the target;
- finding or context favouring the alternative;
- decisive or high-value discriminator;
- important limitation or overlap;
- risk if the alternative is missed.

16.3 Differential priority

Differentials should be prioritised by clinical plausibility, danger, educational importance, and curriculum role.

A rare alternative should not displace a common or dangerous mimic merely because it is more distinctive.

16.4 Relationship governance

Differential text should resolve to governed diagnosis identities where possible.

The presence of a differential in education may support a candidate Teaching Relationship or Differential Link, but authored text does not automatically create a governed graph relationship.

17. Management

17.1 Purpose

The Management section teaches the diagnosis-level principles that a learner should recognise after diagnosis or when urgent action is required.

Management content should remain appropriate to Wardle's educational scope, learner level, clinical context, and evidence base.

17.2 Management structure

Where applicable, management should distinguish:

- immediate stabilisation;
- first-line treatment;
- definitive treatment;
- supportive care;
- management by severity or subtype;
- referral or specialist involvement;
- monitoring;
- follow-up;
- prevention or risk modification;
- management of complications.

17.3 Context and qualification

Management recommendations should identify the context in which they apply.

Statements should not imply universal treatment where decisions depend upon severity, age, pregnancy, comorbidity, local resistance, contraindications, resources, or specialist assessment.

17.4 Drug and dose content

Drug names, routes, doses, intervals, duration, adjustments, and monitoring requirements are high-risk knowledge.

When included, they require exactness, appropriate evidence, population context, unit verification, contraindication awareness, and heightened clinical review.

Dose content should not be generated or copied into learner-facing education without verification against an appropriate current source.

17.5 Local practice and universal principles

Where management varies by jurisdiction, institution, or resource setting, education should distinguish broadly accepted principles from local protocols.

Local practice should not be presented as universal without qualification.

18. Complications

18.1 Purpose

The Complications section teaches important adverse consequences of the disease, delayed recognition, progression, or treatment.

18.2 Complication classification

Where relevant, the section should distinguish:

- direct disease complication;
- severity consequence;
- treatment complication;
- associated condition;
- late sequela;
- recurrence or chronic impairment.

Complications should be prioritised according to seriousness, frequency, preventability, diagnostic value, and educational importance.

18.3 Recognition and prevention

A complication entry should explain how the complication may be recognised, which warning signs matter, and how timely diagnosis or management may reduce risk where appropriate.

Rare catastrophic complications should not be used to sensationalise the resource.

19. Pitfalls

19.1 Purpose

Pitfalls identify plausible learner or clinical reasoning errors and provide a safer governing principle.

A Pitfall should teach an error to avoid rather than merely repeat a fact in negative form.

19.2 Pitfall structure

A strong Pitfall should identify:

- the mistaken assumption or action;
- why it is plausible;
- the consequence;
- the finding or principle that corrects it;
- the safer reasoning or management response.

19.3 Appropriate pitfall types

Pitfalls may address:

- premature closure;
- over-reliance on a single test;
- misinterpretation of a classic sign;
- failure to consider a dangerous mimic;
- incorrect exclusion based on an absent feature;
- unsafe treatment generalisation;
- missed escalation;
- misuse of a scoring system;
- terminology confusion.

Pitfalls should be diagnosis-specific. Generic warnings such as “always correlate clinically” do not satisfy the standard without explaining the relevant decision error.

20. Recall Prompts

20.1 Purpose

Recall Prompts convert governed Diagnosis Education into active-recall units that reinforce clinically meaningful knowledge.

A Recall Prompt is not merely a question appended to a section. It is a governed educational artifact with an identifiable source concept and intended retrieval outcome.

20.2 Prompt standard

A Recall Prompt should:

- test one meaningful concept or a deliberately bounded set of linked concepts;
- derive from governed education;
- have a concise defensible answer;
- include an explanatory layer;
- identify relevant qualifiers;
- match the declared learner level or difficulty;
- avoid ambiguity;
- avoid unsupported extension beyond its source content.

20.3 Prompt families

Recall may test:

- pattern recognition;
- a discriminator;
- an investigation interpretation;
- a scoring-system threshold;
- a management priority;
- a complication warning;
- a pitfall correction;
- a key comparison with a mimic.

20.4 Answer quality

An answer should be specific enough to establish what the learner was expected to retrieve while allowing clinically equivalent wording where appropriate.

Recall answers must not use absolute language where the governed source contains necessary uncertainty or context.

21. References

21.1 Purpose

References identify external sources that support, qualify, or extend Diagnosis Education.

References strengthen traceability and maintenance. They do not substitute for clinical review.

21.2 Reference standard

A reference record should preserve, where available:

- title;
- author or responsible body;
- publisher or journal;
- publication or update date;
- edition or version;
- persistent identifier or stable location;
- date accessed where relevant;
- applicable claim or section;
- jurisdiction or population limitations;
- review status.

21.3 Source selection

Source selection should be proportionate to the claim.

High-risk management, diagnostic criteria, scoring systems, and changing guidelines require stronger source authority and freshness than stable introductory definitions.

21.4 Citation fidelity

A source must not be cited for a claim it does not support.

References must not be fabricated, approximated, or represented as verified when only suggested by an automated system.

Where credible sources disagree, the education should not conceal the disagreement through false certainty.

22. Supplementary Educational Elements

Implementations may support supplementary elements such as definitions, classic-pattern labels, clinical pearls, key concepts, memory aids, or “take-home” summaries.

Supplementary elements do not escape governance because they are brief or visually separated from the principal sections.

They must:

- derive from governed content;
- preserve diagnosis scope;
- avoid contradiction;
- avoid misleading simplification;
- identify evidence needs where clinically material;
- remain versioned with the Diagnosis Education artifact.

A visually prominent callout may carry greater educational influence than a longer paragraph and should be reviewed proportionately.

23. Cross-Section Coherence

Diagnosis Education should read as one governed educational resource rather than a collection of independently generated sections.

Cross-section review should evaluate:

- consistent diagnosis scope;
- consistent terminology;
- compatible severity framing;
- agreement between investigations and differentials;
- agreement between complications and management;
- alignment of pitfalls with the actual content;
- recall prompts that match governed sections;
- references that support the claims they are associated with;
- absence of unnecessary repetition;
- absence of contradictions.

A section change may make another section stale even when that other section has not been edited directly.

Dependency examples

Changing diagnostic criteria may affect the Clinical Pattern, Investigations, Differentials, Pitfalls, and Recall Prompts.

Changing Management may affect Complications, Pitfalls, and high-risk recall content.

Changing diagnosis scope may require review of every section.

24. Diagnosis Specificity and Educational Reasoning

Diagnosis Education must contain meaning that is specific to the governed diagnosis.

Generic phrases may be grammatically acceptable but educationally empty.

Weak content includes:

- “patients may present with a range of symptoms”;
- “investigations are required to confirm the diagnosis”;
- “management depends on severity”;
- “early diagnosis is important”;
- “complications may be serious”;
- “clinical correlation is advised”.

Such statements require diagnosis-specific explanation.

Education should explain why findings matter, how they change suspicion, what they distinguish, what action they prompt, and which error they prevent.

25. Clinical Uncertainty and Qualification

Diagnosis Education should communicate uncertainty accurately.

It should distinguish:

- typical from required;
- supportive from confirmatory;
- common from possible;
- sensitive from specific;
- association from causation;
- diagnosis from severity;
- general principle from local protocol;
- evidence-based recommendation from expert practice.

Absolute terms such as “always,” “never,” “diagnostic,” or “rules out” should be used only where their meaning is clinically defensible within the stated scope.

Uncertainty should not be expressed through vague disclaimers that avoid teaching a clear principle.

26. Learner Level and Cognitive Scope

Diagnosis Education should be appropriate to the intended learner level.

Learner level may influence:

- terminology;
- depth;
- assumed prerequisite knowledge;
- complexity of differentials;
- investigation interpretation;
- management detail;

- use of scoring systems;
- recall difficulty.

Simplification should reduce unnecessary complexity without making the clinical concept inaccurate.

Advanced detail should not obscure the high-yield reasoning principle for a basic learner.

Where the same Diagnosis Education artifact serves multiple levels, layered presentation should preserve a clear core while allowing deeper expansion.

27. Population, Setting, and Equity

Education should represent the populations and settings in which the diagnosis is taught accurately and respectfully.

Where clinically relevant, content should consider:

- age;
- pregnancy;
- sex-related clinical variation;
- comorbidity;
- immunocompromise;
- resource setting;
- geography and epidemiology;
- differences in access to investigation or treatment.

Population descriptors should be used because they change clinical interpretation, not as decorative demographic variation.

Resource limitations should not be ignored, but constrained practice should not be misrepresented as the universal standard of care.

28. Terminology, Units, and Numerical Accuracy

Terminology should align with the governed Diagnosis Registry and accepted clinical usage.

Outdated, ambiguous, stigmatising, or locally idiosyncratic terminology should be avoided or explained.

Numerical content requires particular care.

Values should preserve:

- correct units;
- reference context;
- age or population variation;
- threshold meaning;
- conversion accuracy;
- significant clinical qualifiers;

- source version where values may change.

A unit or decimal error may transform otherwise correct education into high-risk misinformation and must be treated accordingly.

29. Completeness and Non-Applicability

Completeness means that the artifact addresses the educational requirements applicable to the diagnosis and intended use.

Completeness does not require meaningless text in every possible section.

A section may be marked not applicable where:

- the concept genuinely does not apply;
- the artifact scope excludes it deliberately;
- the learner level does not require it;
- another governed artifact carries the content and the relationship is explicit.

Non-applicability should be an editorial conclusion, not a substitute for missing authoring or failed generation.

Generic filler must not be used to create the appearance of completeness.

30. Diagnosis Education Lifecycle

The canonical Diagnosis Education lifecycle should preserve the distinction between production, review, approval, publication, maintenance, and archival states.

Current conceptual progression:

Lifecycle

Draft or Generated -> Needs Review -> Needs Edit or Approved -> Published -> Maintained -> Archived, Superseded, or Withdrawn

Lifecycle labels may differ by implementation, but their editorial meaning must remain explicit.

30.1 Draft

The artifact is under human authoring or controlled revision and remains candidate knowledge.

30.2 Generated

The artifact or section has been produced primarily by an automated system and has not yet received the required human review.

Generated status must not be treated as evidence of completeness, correctness, or readiness.

30.3 Needs Review

The artifact is sufficiently formed for editorial evaluation but no final approval has been granted.

30.4 Needs Edit

Material concerns prevent approval. Required changes should remain linked to explicit review findings.

30.5 Approved

The identified revision has satisfied its required editorial review.

Approval does not automatically establish publication, learner exposure, or continued validity after later material change.

30.6 Published

An approved revision has received explicit publication authorisation and is available within its intended learner-facing context.

30.7 Rejected

The proposed artifact or revision has not been accepted for governed use.

Rejection should preserve rationale and should not erase the draft or its provenance.

30.8 Archived, Superseded, or Withdrawn

The artifact is no longer the active learner-facing resource but remains historically preserved according to governance policy.

31. Review Standard

Diagnosis Education review should evaluate the complete artifact and, where necessary, its material sections separately.

Review dimensions include:

- diagnosis identity and scope;
- clinical correctness;
- educational purpose;
- diagnosis specificity;
- cross-section coherence;
- differential reasoning;
- investigation interpretation;
- management safety;
- evidence sufficiency;
- learner level;
- terminology;
- recall quality;

- publication readiness.

Review should identify findings and rationale rather than silently editing the artifact into an approved state.

32. Clinical Review

Clinical review should determine whether Diagnosis Education is accurate, appropriately qualified, safe, and consistent with accepted practice within its stated context.

Clinical review should pay particular attention to:

- diagnostic criteria;
- exclusion statements;
- investigation limitations;
- drug doses;
- emergency management;
- paediatric and pregnancy-related content;
- procedural guidance;
- contraindications;
- escalation thresholds;
- evolving guidelines.

Review intensity should reflect the consequence of error.

33. Educational Review

Educational review should determine whether the artifact teaches the intended knowledge clearly, efficiently, and coherently.

It should assess:

- alignment with Learning Goals;
- prioritisation;
- clarity;
- reasoning value;
- avoidance of trivia;
- appropriate cognitive load;
- usefulness of comparisons;
- quality of pitfalls;
- quality and difficulty of recall;
- duplication across sections.

An artifact should not be approved merely because every section contains text.

34. Evidence Review

Evidence review should determine whether material claims are appropriately supported and whether sources remain relevant to the stated diagnosis, population, setting, and time.

Evidence review should be heightened for:

- drug doses;
- treatment duration;
- diagnostic criteria;
- scoring systems;
- time-sensitive interventions;
- screening recommendations;
- guideline-dependent management;
- claims likely to change with emerging evidence.

Where evidence is incomplete or contested, the education should communicate a defensible bounded conclusion rather than imply certainty.

35. Approval Standard

Approval applies to an identified Diagnosis Education revision.

Approval should require:

- resolved diagnosis identity;
- defined scope;
- required sections complete or deliberately non-applicable;
- no unresolved blocking clinical defects;
- no unresolved high-risk unsupported claims;
- appropriate clinical and educational review;
- acceptable cross-section coherence;
- adequate evidence for material claims;
- complete provenance and review records.

Conditional approval may be used only where governance policy permits and where remaining obligations do not create unsafe learner exposure.

36. Publication Standard

Publication is separate from approval.

A Diagnosis Education revision may be approved but not published because diagnosis-level dependencies, publication strategy, learner scope, or operational conditions remain unresolved.

Publication assessment should consider:

- approved revision;
- publication channel;
- intended learner population;
- readiness blockers;
- evidence freshness;
- consistency with published cases;
- maintenance ownership;
- effect of replacing an earlier published revision.

Technical availability must not be interpreted as publication authority.

37. Section-Level Drafting, Repair, and Regeneration

A specific section may be drafted, repaired, or regenerated without replacing the complete Diagnosis Education artifact.

Section-level work should preserve:

- target section;
- source artifact revision;
- original section;
- reason for change;
- input context;
- proposed replacement;
- affected dependencies;
- review decision;
- resulting artifact revision.

A section must not be applied solely because it is locally improved. Review must consider whether it remains coherent with the complete artifact.

Published content should not be overwritten directly by a generated replacement.

38. Relationship to Clinical Cases

Clinical Cases and Diagnosis Education should reinforce the same governed diagnosis foundation while serving different educational functions.

Cases may contribute examples, reasoning insights, discriminators, and observed misconceptions to Diagnosis Education maintenance.

Diagnosis Education may provide context, differentials, investigations, management, and recall concepts that cases reinforce.

The relationship should not require every case to teach every section or every education section to appear within a case explanation.

Material contradiction between a published case and published Diagnosis Education should create a governance issue requiring review.

39. Relationship to Knowledge and Reasoning Governance

Diagnosis Education both consumes and produces knowledge-governance signals.

Governed Graph Facts, Teaching Relationships, Evidence Relationships, Reasoning Paths, and Differential Links may inform education authoring and review.

Education may also produce Graph Candidates such as findings, investigations, mimics, pitfalls, management principles, complications, recall concepts, and reasoning statements.

Extraction preserves provenance. It does not automatically transform learner-facing prose into a governed fact.

40. Relationship to Artificial Intelligence

Artificial Intelligence may generate complete Diagnosis Education drafts, draft specific sections, identify omissions, compare revisions, propose repairs, generate recall prompts, and assist maintenance.

AI-generated education remains candidate knowledge until human review and controlled application.

AI generation should use governed context including:

- diagnosis identity;
- Editorial Brief;
- Learning Goals;
- Teaching Rules;
- required mimics and discriminators;
- investigation and management anchors;
- complications and pitfalls;
- learner level;
- evidence requirements;
- current education revision.

AI must not create institutional trust, approve its own output, fabricate references, or publish education independently.

Acceptance of an AI Draft approves the proposal for controlled application. It does not itself approve the resulting Diagnosis Education revision.

41. Validation

Automated and structured validation may support Diagnosis Education governance by detecting defects that can be evaluated deterministically or consistently.

Validation may assess:

- required structure;
- diagnosis mapping;
- empty sections;
- generic filler;
- duplicate content;
- terminology consistency;
- unit format;
- broken references;
- unsupported absolute language;
- recall answer completeness;
- cross-section conflicts;
- stale dependencies;
- publication prerequisites.

Validation does not establish clinical correctness or educational quality independently of human review.

Validation results should remain version-specific and become stale after material change.

42. Maintenance and Revalidation

Published Diagnosis Education remains living governed knowledge.

Maintenance may be triggered by:

- new evidence;
- updated guideline;
- changed diagnostic criteria;
- changed scoring system;
- terminology change;
- new treatment safety information;
- learner feedback;
- case contradiction;
- quality audit;
- specialty review;
- scheduled revalidation.

Maintenance should assess both the changed claim and the sections that depend upon it.

High-risk or rapidly evolving content should have shorter review intervals than stable foundational content.

Failure to complete required maintenance may block continued publication or require temporary withdrawal.

43. Withdrawal, Supersession, and Archival

Diagnosis Education may be withdrawn where continued learner exposure is unsafe, misleading, outdated, or no longer institutionally supported.

A revised artifact may supersede an earlier published revision while preserving historical access for governance and audit.

Archival should preserve:

- artifact and revision identity;
- publication history;
- reason for withdrawal or supersession;
- replacement artifact where applicable;
- responsible authority;
- affected recall prompts and extracted knowledge;
- learner-facing correction requirements where applicable.

Removal from active use must not erase governance history.

44. Quality Assessment

Diagnosis Education quality should be assessed through a combination of editorial review, validation, maintenance signals, and learner-informed evidence.

Quality indicators may include:

- clinical defect rate;
- unsupported-claim rate;
- cross-section contradiction rate;
- generic-content rate;
- learning-goal coverage;
- differential and discriminator coverage;
- evidence freshness;
- recall ambiguity;
- post-publication correction rate;
- learner-reported confusion;
- maintenance backlog.

Learner performance may identify possible educational weakness. It must not independently determine clinical correctness.

45. Implementation Requirements

Implementation should preserve the canonical distinctions established in this Part regardless of database design or interface layout.

The system should support:

- stable Diagnosis Education identity;
- versioned revisions;
- section-level identity where material governance requires it;
- diagnosis mapping;
- source provenance;
- draft separation;
- human review;
- artifact-level and section-level findings;
- cross-section dependency tracking;
- explicit approval;
- separate publication decision;
- withdrawal and supersession;
- recall linkage;
- reference linkage;
- knowledge extraction provenance;
- maintenance triggers;
- audit history.

Where current implementation differs from the Canon, the divergence should be documented explicitly rather than allowed to redefine the artifact silently.

46. Normative Statements

The following statements summarise the mandatory requirements of this Part.

- Every Diagnosis Education artifact **MUST** resolve to a governed Diagnosis Registry identity.
- Diagnosis Education **MUST** remain distinct from a Case Explanation.
- Approval and publication **MUST** apply to an identified artifact revision.
- Generated education **MUST** remain candidate knowledge until human review and controlled application.
- A section marked non-applicable **MUST** represent an editorial conclusion rather than missing work.
- Diagnosis Education **MUST** be evaluated for clinical correctness and educational usefulness.
- Material sections **MUST** remain coherent with the complete artifact.

- High-risk management, dose, criteria, and scoring content MUST receive proportionate clinical and evidence review.
 - Differentials MUST teach clinically meaningful comparison rather than provide an unprioritised list.
 - Recall Prompts MUST derive from governed education and preserve necessary qualifiers.
 - References MUST NOT be fabricated or cited for claims they do not support.
 - Automated validation MUST NOT substitute for human clinical or educational judgement.
 - Publication MUST remain separate from approval and technical availability.
 - Material change MUST create a new governed revision or equivalent version-specific record.
 - Withdrawal, supersession, and archival MUST preserve governance history.
 - Current implementation divergence from this Canon MUST be recorded explicitly.
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47. Relationship to the Wider Canon

Part I defines the foundational distinctions and universal artifact standard governing Diagnosis Education.

Part II defines the diagnosis identity, Editorial Brief, Learning Goals, Teaching Rules, and education guidance from which Diagnosis Education derives its purpose.

Part III defines the Clinical Cases and Case Explanations that Diagnosis Education complements without duplicating.

Part V defines the Graph Facts, Evidence Relationships, Teaching Relationships, Differential Links, Reasoning Paths, and extraction governance that support or emerge from Diagnosis Education.

Part VI defines how Artificial Intelligence may generate or revise Diagnosis Education while remaining subordinate to human authority.

Part VII defines the assessments, reviews, decisions, validation results, publication records, and governance history through which Diagnosis Education becomes institutionally trusted.

Together, these Parts ensure that learner-facing diagnosis education is not merely produced, but clinically governed, educationally coherent, explainable, maintainable, and safe for institutional publication.

Closing Principle

Diagnosis Education should not be judged by how much information it contains.

It should be judged by whether it helps learners recognise the diagnosis, interpret evidence, distinguish important alternatives, act safely, avoid predictable errors, and retain the knowledge that matters.

Within WEOS:

- Educational intent defines what should be taught.
- Diagnosis Education communicates that intent.
- Clinical and educational review establish confidence.
- Evidence supports material claims.
- Publication authorises learner exposure.
- Maintenance preserves trust as knowledge evolves.